



METOCLODE SYRUP 5MG/5ML

Description: Plain, pineapple flavoured syrup.

Contents: Per 5ml:

Metoclopramide HCl	5.0mg
Methylparaben	0.1 % w/v
Propylparaben	0.01 % w/v

Indications:

Adult population

Metoclopramide is indicated in adults for:

- Prevention of delayed chemotherapy induced nausea and vomiting (CINV).
- Prevention of radiotherapy induced nausea and vomiting (RINV).
- Symptomatic treatment of nausea and vomiting, including acute migraine induced nausea and vomiting.

Pediatric population

Metoclopramide is indicated in children (aged 1-18 years) for prevention of delayed chemotherapy induced nausea and vomiting (CINV) as a second line option.

Pharmacology:

Metoclopramide hydrochloride has central and anti-emetic properties. It also promotes gastric motility and accelerates gastric emptying by its dopamine antagonist action. Oral absorption is rapid and it undergoes first-pass hepatic metabolism and renal excretion.

Adverse effects:

Very common-somnolence, asthenia, extrapyramidal disorders (particularly in children and young adults and/or when the recommended dose is exceeded, even following administration of a single dose of the drug, parkinsonism, akathisia, depression, restlessness and hypotension (particularly with intravenous formulation). Uncommon-bradycardia (particularly with intravenous formulation), amenorrhoea, hyperprolactinaemia, hypersensitivity, dystonia, dyskinesia, depressed level of consciousness and hallucination. Rare-galactorrhoea, constipation, nausea, unusual dryness of mouth, oedema (including face oedema), convulsion especially in epileptic patients, dizziness, headache, confusional state, trouble sleeping, unusual irritability, skin rash, urticaria and pruritus. Others include methaemoglobinemia, which could be related to NADH cytochrome b5 reductase deficiency, particularly in neonates, sulfaemoglobinemia, mainly with concomitant administration of high doses of sulfur-releasing medicinal products, cardiac arrest, occurring shortly after injectable use, and which can be subsequent to bradycardia, asystole, atrioventricular block, sinus arrest particularly with intravenous formulation, electrocardiogram QT prolonged, Torsade de Pointes, gynecomastia, visual disturbances, anaphylactic reaction (including anaphylactic shock particularly with intravenous formulation), tardive dyskinesia which may be persistent, during or after prolonged treatment, particularly in elderly patients, neuroleptic malignant syndrome, dyspnea, shock and acute hypertension in patients with pheochromocytoma have been reported. Endocrine disorder during prolonged treatment in relation with hyperprolactinaemia (amenorrhoea, galactorrhoea, galactorrhoea, gynecomastia). The following reactions sometimes associated, occur more frequently when high doses are used.

- Extrapyramidal symptoms: acute dystonia and dyskinesia, parkinsonian syndrome, akathisia, even following administration of a single dose of the medicinal product, particularly in children and young adults.

- Drowsiness, decreased level of consciousness, confusion, hallucination.

Effects on ability to drive and use machine:

Metoclopramide may cause drowsiness, dizziness, dyskinesia and dystonia which can affect the vision and also interfere with the ability to drive and operate machinery.

Warnings and Precautions:

- Avoid doses exceeding 0.5mg/kg/day.
- Extrapyramidal effects, especially dystonic reaction of metoclopramide are more likely to occur in children shortly after initiation of therapy, and usually with doses higher than 0.5mg per kg of weight per day.
- Caution in patients taking other medications that can cause extrapyramidal reactions, such as phenothiazines.
- Caution if taking other CNS depressants.

Neurological disorders: Extrapyramidal disorders may occur, particularly in children and young adults, and/or when high doses are used. These reactions generally occur at the beginning of treatment, and can occur after a single dose, if extrapyramidal symptoms occur, metoclopramide should be discontinued immediately. These effects are generally completely reversible after treatment discontinuation; however, symptomatic treatment may be required (benzodiazepines in children, and/or anticholinergic antiparkinsonian medicinal products in adults). An interval of at least six hours should be respected between each dose even if vomiting or rejection of the dose occurs, in order to avoid overdose.

Long-term treatment with metoclopramide may cause potentially irreversible tardive dyskinesia particularly in elderly subjects. Treatment should not exceed 3 months because of the risk of tardive dyskinesia. Treatment must be discontinued if clinical signs of tardive dyskinesia occur. Neuroleptic malignant syndrome has been described with metoclopramide in combination with neuroleptics and with metoclopramide monotherapy. Metoclopramide must be immediately discontinued if symptoms of neuroleptic malignant syndrome develop, and appropriate treatment should be initiated.

Particular caution should be exercised in patients with underlying neurological disorders, and in patients receiving other centrally-acting drugs.

Symptoms of Parkinson's disease may also be exacerbated by metoclopramide.

Methaemoglobinemia: Methaemoglobinemia, which could be related to NADH cytochrome b5 reductase deficiency, has been reported. If this occurs, treatment must be immediately and permanently discontinued, appropriate measures initiated (such as treatment with methylene blue).

Cardiac disorders: Serious cardiovascular undesirable effects, including cases of severe bradycardia, circulatory collapse, cardiac arrest and QT prolongation have been reported during administration of metoclopramide by injection, particularly via the intravenous route. Particular caution should be exercised when administering metoclopramide, particularly via the intravenous route, in elderly subjects, patients with cardiac conduction disorders (including QT prolongation), patients with electrolyte imbalance, bradycardia, and patients taking other drugs known to prolong QT interval.

The intravenous injection must be given as a slow bolus (of at least 3 minutes duration) in order to reduce the risk of undesirable effects (e.g. hypotension, akathisia).

Kidney or liver failure: In patients with kidney failure or severe liver failure, a dose reduction is recommended.

Pregnancy and Lactation:

A large amount of data on pregnant women (more than 1000 exposed outcomes) indicates neither malformative toxicity nor fetotoxicity. Metoclopramide can be used during pregnancy if clinically needed. Due to pharmacological properties (as with other neuroleptics), in case of metoclopramide administration at the end of pregnancy, extrapyramidal syndrome in newborn cannot be excluded. Metoclopramide should be avoided at the end of pregnancy. If metoclopramide is used, neonatal monitoring should be undertaken.

Metoclopramide is excreted in breast milk at low level. Adverse reactions in the breast-fed baby cannot be excluded. Therefore, metoclopramide is not recommended during breastfeeding. Discontinuation of metoclopramide in breast-feeding women should be considered.

Drug Interactions:

Alcohol-potentiates the sedative effect of metoclopramide, concurrent use may accelerate gastric emptying of alcohol and may promote the rate and extent of



absorption from the small intestine. Anticholinergics and morphine derivatives may have both a mutual antagonism with metoclopramide or the digestive tract motility. Central nervous system depressants (morphine derivatives, anxiolytics, sedative H1 antihistamines, sedative antidepressants, barbiturates, clonidine and related) -sedative effects of central nervous system depressants and metoclopramide are potentiated.

Neuroleptics-Metoclopramide may have an additive effect with other neuroleptics on the occurrence of extrapyramidal disorders.

Serotonergic drugs-The use of metoclopramide with serotonergic drugs such as SSRIs may increase the risk of serotonin syndrome.

Digoxin-Metoclopramide may decrease digoxin bioavailability. Careful monitoring of digoxin plasma concentration is required.

Ciclosporin-Metoclopramide increases ciclosporin bioavailability (Cmax by 46% and exposure by 22%). Careful monitoring of ciclosporin plasma concentration is required. The clinical consequence is uncertain.

Mivacurium and suxamethonium-Metoclopramide injection may prolong the duration of neuromuscular block (through inhibition of plasma cholinesterase).

Strong CYP2D6 inhibitors-Metoclopramide exposure levels are increased when co-administered with strong CYP2D6 inhibitors such as fluoxetine and paroxetine.

Although the clinical significance is uncertain, patients should be monitored for adverse reactions.

Extrapyramidal reaction causing drugs (such as phenothiazines and tetraabenazine)-Concurrent use with metoclopramide may increase the frequency and severity of extrapyramidal side effects. Care should be exercised in the event of co-administration of these drugs.

Mexiletine-Concurrent use with metoclopramide may accelerate absorption of mexiletine. Diagnostic interference-With Gonadorelin test, concurrent use with metoclopramide may blunt the response to gonadorelin by increasing serum prolactin concentrations. Concurrent metoclopramide therapy may increase aldosterone and serum prolactin levels.

Aspirin and paracetamol-The absorption of these concurrently administered oral drug may be modified by the effect of metoclopramide on gastric motility.

Alovesque-Metoclopramide may reduce plasma concentrations of alovesque.

Contraindications:

- Hypersensitivity to the active substance or to any of the excipients listed.
- Gastrointestinal haemorrhage, mechanical obstruction or gastro-intestinal perforation for which the stimulation of gastrointestinal motility constitutes a risk.

- Confirmed or suspected pheochromocytoma, due to the risk of severe hypertension episodes.

- History of neuroleptic or metoclopramide-induced tardive dyskinesia.

- Epilepsy (increased crises frequency and intensity).

- Parkinson's disease.

- Combination with levodopa or dopaminergic agonists.

- Known history of methemoglobinemia with metoclopramide or of NADH cytochrome-b5 deficiency.

- Use in children less than 1 year of age.

Dosage And Administration (oral):

Adults: The recommended single dose is 10mg repeated up to three times daily. The maximum recommended daily dose: 30mg or 0.5mg/kg body weight. Total daily dose of metoclopramide, especially for children and young adults, should not normally exceed 0.5mg/kg body weight.

The maximum recommended treatment durations is 5 days.

Prevention of delayed chemotherapy induced nausea and vomiting (CINV) (paediatric patients aged 1-18 years)

The recommended dose is 0.1 to 0.15 mg/kg body weight, repeated up to three times daily by oral route. The maximum dose in 24 hours is 0.5mg/kg body weight.

Dosing table

Age	Body Weight	Dose	Frequency
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1-3 years	10-14kg	1mg	Up to 3 times daily
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3-5 years	15-19kg	2mg	Up to 3 times daily
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5-9 years	20-29kg	2.5mg	Up to 3 times daily
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9-18 years	30-60kg	5mg	Up to 3 times daily
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15-18 years	Over 60kg	10mg	Up to 3 times daily
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The maximum treatment duration is 5 days for prevention of delayed chemotherapy induced nausea and vomiting (CINV).

Tablets are not suitable for use in children weighing less than 30kg. Other pharmaceutical forms may be more acceptable for administration to this population.

Frequency of administration:

A minimum interval of 6 hours between two administrations is to be respected, even if vomiting or rejection of the dose occurs.

Special population

Elderly:

In elderly patients, a dose reduction should be considered, based on renal and hepatic function and overall frailty.

Renal impairment:

In patients with end stage renal disease (Creatinine clearance ≤ 15 ml/min), the daily dose should be reduced by 75%.

In patients with moderate to severe renal impairment (Creatinine clearance 15-50ml/min), the daily dose should be reduced by 50%.

Hepatic impairment:

In patients with severe hepatic impairment, the dose should be reduced by 50%.

Overdose

Symptoms: Extrapyramidal disorder (muscle spasms, especially of jaw, neck, back, shuffling walk, tic like (jerky) movements of head and face, trembling and shaking of hands), drowsiness, decreased level of consciousness, confusion, hallucination and cardio-respiratory arrest may occur.

Management: In case of extrapyramidal symptoms related or not to overdose, the treatment is only symptomatic (benzodiazepines in children and/or anticholinergic anti-parkinsonian medicinal products in adults). A symptomatic treatment and a continuous monitoring of the cardiovascular and respiratory functions should be carried out according to clinical status.

Storage conditions: Store in a cool, dry place below 30°C. Protect from light. Keep away from children.

Presentation: Syrup 5mg/5ml x 60ml

Shelf life: 2 years

Product Registration: PRIME PHARMACEUTICAL SDN. BHD.

Holder & Manufacturer: 1505, Lorong Perusahaan Utama 1, Taman Perindustrian Bukit Tengah,

14000 Bukit Mertajam, Penang, Malaysia

15/05/2026